AT 50/75

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

AT 50/75 · treatment-day support

Active medicines: Doxorubicin + Docetaxel

Emetogenic risk: Moderate · doxorubicin50 with docetaxel

Medicine	Dose / route	When
Ondansetron	8 mg orally twice daily	Day 1 NCCP alternative to 16 mg once daily; review QT / hepatic risks
Dexamethasone	8 mg orally	Day 1; reconcile docetaxel premedication Do not add to the docetaxel steroid course automatically
Dexamethasone	8 mg orally daily	Days 2–3 reference antiemetic row Reconcile the delayed period with docetaxel steroids; one final individualised steroid prescription
Metoclopramide	10 mg orally up to three times daily as required	Subsequent days: duration to be confirmed Review contraindications and renal function

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before chemotherapy; at least 3 doses before the infusion. If missed, contact the team; any intravenous (IV) replacement is a consultant decision.
- Reconcile the total dexamethasone schedule with antiemetics, including aprepitant interactions. The listed antiemetic steroid is a reference row, not an instruction to add a second course.
- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to the exact regimen and individual risk.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00423 v5 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-23 · SACTCheck v0.84.17 · Regimen-level drafts; prescriber and oncology-pharmacy verification required